Supplementary Table 1. Demographic, clinical presentation, comorbid conditions, lesion site, treatment, and outcome data of patients with jejunal Dieulafoy's lesion (1944–2010).

Authors and year of publication	Country	Age/ gender	Clinical presentation	Hemoglobin (g/dL)	Comorbid conditions	AOC/ NSAIDs	Diagnostic workup	Diagnostic attempts	Treatment	Rebleed/ treatment	Outcome, follow-up duration
Levine and Valk, 1944 [11]	USA	18/F	Abdominal pain, hematochezia	5	None	No	UGI series was unremarkable. Ex-lap showed proximal jejunal mass filled with thrombus that actively bled on manipulation *	2	Surgical resection	No	Recovered
Rathmell et al. 1951 [14]	USA	29/F	Massive rectal bleeding	NR	None	No	Ex-lap failed. Autopsy showed 5-mm aneurysmal artery in second part of the jejunum	-	-	-	Died
Gueco et al. 1969 [15]	USA	20/F	Melena	NR	None	No	Laparotomy showed the jejunal lesion, 15-20 cm from the ligament of Treitz	1	Surgical resection	No	Recovered
Matuchansky et al. 1978 [16]	France	31/M	Shock, BRBPR, melena	6	None	No	IOE failed. SMA angiography detected the lesion in proximal jejunum. Repeat ex-lap confirmed the lesion *	3	Surgical resection	No	Recovered, 48 months
Matuchansky et al. 1978 [16]	France	24/M	Melena	8	Childhood asthma	No	EGD, colonoscopy, and small bowel X-ray failed. Ex-lap revealed lesion in third jejunal loop *	4	Surgical resection	No	Recovered, 12 months
Boix et al. 1988 [17]	Spain	50/F	Melena	NR	Marked anemia	NR	EGD, colonoscopy, selective celiac angiography, and X-ray failed. Bleeding scan detected and IOE confirmed the jejunal lesion at 100 cm from the ligament of Treitz *	3	Surgical resection	No	Recovered, 9 months
Park et al. 1988 [44]	Korea	55/F	Hematemesis, dizziness, epigastric pain	8.3	Subtotal gastrojejunostomy	NR	Gastric lavage failed. Gastroscope showed a small exposed bleeding vessel at the jejunal side of the anastomosis	2	Epinephrine + photocoagulation	No	Recovered
Vetto et al. 1989 [54]	USA	58/M	Shock, BRBPR, abdominal pain	NR	Intravenous drug abuse, HTN, HOCM, CHF, TB endocarditis, pericarditis, hepatitis	No	EGD and colonoscopy failed. Angiography indicated midjejunal dye extravasation. Ex-lap showed mass lesion with artery at 20 cm from the ligament of Treitz *	4	Surgical resection	No	Recovered, 6 months
Vetto et al. 1989 [54]	USA	72/M	Melena	NR	Aortic insufficiency, vagatomy, pyloroplasty	No	UGI series, barium enema, visceral angiography, and ex-lap failed. Repeat ex-lap showed large submucosal vessel at 45 cm distal to the ligament of Treitz	5	Surgical resection	No	Recovered
Katz et al. 1991 [103]	USA	16/M	Orthostatic weakness, hematochezia	9	Dyslexia	No	SM angiogram pinpointed small bowel bleeding. Ex-lap revealed a 1-cm lesion in the distal jejunum *	2	Surgical resection	No	Recovered
Saunders et al. 1991 [39]	UK	34/M	BRBPR, shock	NR	None	Ibuprofen	EGD failed. ^{99}Tc-labelled pinpointed and selective red cell scan identified jejunal lesion. Ex-lap confirmed a large solitary submucosal proximal jejunal artery	4	Surgical resection	No	Recovered
Goins et al. 1995 [23]	USA	36/M	Shock, hematochezia	NR	Drug abuse (cocaine, heroin), diverticulosis	No	Rigid proctosigmoidoscopy, blood scan, and IOE failed even after subtotal colectomy. Mesenteric angiography twice indicated but couldn't pinpoint the lesion. Repeat IOE showed an antimesenteric actively bleeding lesion, 30 cm distal to the ligament of Treitz *	6	Surgical resection	No	Recovered
Murray et al. 1996 [89]	USA	12/M	Hematemesis, hematochezia	13	Congenital heart disease, prior GI obstruction	Aspirin	EGD showed 2-3 mm protruding artery from a nonulcerated mucosal surface inside the efferent limb of small bowel	1	Epinephrine + endoscopic band ligation	No	Recovered, 4 months
Lee et al. 1997 [5]	Korea	20/F	Hematochezia	8.1	None	No	EGD and colonoscopy failed. Blood scan detected lesion involving third branch of jejunal artery, which was confirmed by SMA angiography and ex-lap *	4	Surgical resection	No	Recovered, 2 months
Gadenstätter et al. 1998 [53]	Austria	54/M	Hematemesis, melena,	7	Billroth II gastrectomy, GIB	No	EGD revealed a spurting arterial vessel in the efferent limb of the jejunum, confirmed by surgery *	1	Injection sclerotherapy	Yes/ surgery	Recovered, 72 months
Gadenstätter et al. 1998 [53]	Austria	45/M	Hematemesis, melena,	7.2	Billroth II gastrectomy	No	EGD showed a pulsating vessel without active hemorrhage in the efferent jejunal limb	1	Injection sclerotherapy	Yes/ surgery	Recovered, 36 months
Deutsch et al. 1998 [35]	USA	27/M	Hematochezia	NR	NR	No	Diagnostic evaluation failed to identify the source on initial admission. Ex-lap identified a proximal jejunal submucosal aneurysm on the mesenteric border *	3	Surgical resection	No	Recovered
Geschwind et al. 1998 [105]	USA	73/M	Acute GIB	NR	Ileal resections for GIB	NR	Tc-99m RBC scintigraphy was positive for jejunal lesion but celiac angiography was negative, laparotomy with portable intraoperative Tc-99m RBC scintigraphy confirmed *	4	Surgical resection	No	Recovered
Al Asseeri et al. 2000 [30]	KSA	64/M	Epigastric discomfort,	12.6	HTN	No	EGD, colonoscopy and Meckel's nuclear scan failed. Red cell scan detected bleeding	5	Surgical resection	No	Recovered, 6 months

			dizziness, fatigue, melena				focus in cecum and colon. Ex-lap showed a feeding vessel traversing the mesentery into the jejunal lesion *		(RH+ JSR)		
Ertan et al. 2000 [34]	USA	70/F	Obscure, recurrent, massive GIB	NR	NA	NR	EGD, colonoscopy, enteroscopy with pediatric colonoscope, and angiography failed. PE showed single lesion in the proximal jejunum	5	Epinephrine + ethanolamine oleate injection	No	Recovered, 12 months
Fallows et al. 2000 [51]	Canada	19/F	BRBPR	10	D&C for DUB, intestinal intussusception	Naproxen	EGD, colonoscopy, CTA, CT abdomen and small bowel barium follow-through failed. Ex-lap revealed a mid-jejunal intussuscepted segment *	6	Surgical resection	No	Recovered 18 months
Nikolaidis et al. 2001 [91]	Greece	28/M	Melena	9.2	GIB, pyloric stenosis surgery	Yes	Upper endoscopy identified jejunal lesion	1	Endoscopic band ligation	Yes/ surgery	Recovered
Blecker et al. 2001 [36]	USA	18/M	Episodic syncope, melena	NR	None	Acetaminophen	EGD, colonoscopy, and initial blood scan failed. Repeat red cell scan identified the lesion. IOE through ex-lap a small, protuberant vessel in the proximal jejunum *	4	Surgical resection	No	Recovered
Metyas et al. 2001 [37]	USA	41/M	Massive rectal bleeding	12.2	Hypercholesterolemia	No	Bleeding scan, EGD, colonoscopy, and sigmoidoscopy failed to pinpoint the lesion. CTA showed 2 apparent aneurysms on the jejunal branch of SMA. Ex-lap identified the lesion *	6	Jejunal resection + Meckel's diverticulum excision	No	Recovered, 6 months
Owaki et al. 2002 [104]	Japan	12/F	Hematochezia	NR	None	No	EGD and colonoscopy failed. CTA revealed pulsatile extravasation with pseudoaneurysmal dilatation in the distribution of distal jejunal branch *	4	Surgical resection	No	Recovered, 24 months
Ueno et al. 2002 [38]	Japan	37/M	Hematochezia, hypotension	9.6	GIB	No	CTA, EGD, and colonoscopy were unremarkable. IOE showed a 6-mm submucosal lesion in the jejunum, 40 cm distal to the ligament of Treitz	4	Surgical resection	No	Recovered, 96 months
Wan Muhaizan et al. 2002 [22]	Malaysia	27/M	Giddiness, melena, syncope	NR	None	No	EGD failed. Ex-lap showed a small nodule on the jejunal mucosa located 40 cm from duodenojejunal junction was identified through palpation from the serosal aspect *	2	Surgical resection	No	Recovered, 24 months
Mino et al. 2004 [106]	Japan	31/F	Spontaneous LOC, melena	NR	None	No	EGD, colonoscopy, X-ray, USG, CT, and Meckel's scintigraphy failed. Red cell scan and CTA revealed a jejunal bleeding source in the area between the first and second jejunal branches. Ex-lap confirmed the lesion *	9	Surgical resection	No	Recovered, 36 months
Baker et al. 2005 [66]	USA	77/F	Anemia, melena weakness	6	NR	NR	EGD, colonoscopy, 3 PE, CE, and CTA failed. Blood scan showed jejunal bleeding. Ex-lap with IOE identified the proximal jejunal lesion *	11	Surgical resection	No	Recovered
Kim et al. 2005 [25]	Korea	35/M	Dizziness, palpitation, indigestion	9.1	None	No	EGD, colonoscopy, and angiography failed. CT showed a mass in the lumen of the jejunum. Enteroclysis revealed a jejunal stricture with central ulceration. Ex-lap confirmed the diagnosis *	6	Surgical resection	No	Recovered
Katsinelos et al. 2005 [78]	Greece	72/M	Hematemesis, melena	9.6	HTN, angina	Yes	Endoscopy showed visible jejunal vessel	1	Epinephrine + dextrose 50%	Yes/ surgery	Died
Ke et al. 2006 [57]	China	67/F	Abdominal pain, melena	NR	Maltoma	No	CT didn't reveal the lesion. Angiogram showed mid-jejunal bleeder	3	TAE	Yes/ surgery	Recovered
Nga et al. 2007 [45]	China	39/M	Dysentery, postural giddiness, lethargy	7.4	None	No	EGD and colonoscopy unremarkable. CTA localized jejunal lesion. Ex-lap showed active jejunal bleeding from a 1-cm large reddish, 30 cm from the duodenojejunal junction	4	Surgical resection	No	Recovered
Sai Prasad et al. 2007 [107]	Singapore	13/M	Giddiness	7.5	None	No	EGD and blood scan failed. CE showed a bleeding solitary jejunal polyp, from around 100 cm from the duodenojejunal junction. Ex-lap confirmed the diagnosis *	4	Surgical resection	No	Recovered, 12 months
Iacopini et al. 2007 [81]	Italy	70/M	Acute GIB	8	None	No	Endoscopy revealed the lesion in proximal segment of the efferent jejunal loop	2	APC	No	Recovered, 45 months
Yano et al. 2008 [95]	Japan	72/F	Obscure GIB	NR	NR	NR	DBE showed a punctulate lesion with pulsatile bleeding was found at the proximal jejunum	1	Endoscopic hemoclipping	No	Recovered
Yano et al. 2008 [95]	Japan	56/M	Obscure GIB	NR	NR	NR	DBE showed a reddish protruding lesion with a whitish clot was found at the jejunoileal junction	1	Endoscopic hemoclipping	No	Recovered
Pérez-Roldán et al. 2009 [46]	Spain	15/F	BRBPR, shock	NR	None	No	EGD and colonoscopy failed. Ex-lap with IOE showed proximal jejunal lesion *	3	Surgical resection	No	Recovered
Rim 2009 [69]	USA	30/M	BRBPR, fatigue	10.7	None	No	X-ray, bleeding scan and angiography failed. Surgery identified proximal jejunal mass *	4	Surgical resection	No	Recovered
Moreira-Pinto et al. 2009 [12]	Portugal	14/F	Nausea, vomiting, dizziness, LOC, hematochezia	10	None	No	EGD, colonoscopy, and CTA failed. Ex-lap showed bleeding lesion at 14 cm distal to the Treitz angle *	4	Surgical resection	No	Recovered, 24 Months
Yang et al.	Taiwan	54/M	Melena,	3.3	DJ	No	EGD and colonoscopy failed. CT identified	5	Endoscopic	No	Recovered

2009 [86]			syncope, palpitation, DOE		diverticulosis		diverticulosis and CTA detected the jejunal lesion. Peroral DBE confirmed an actively bleeding Dieulafoy-like lesion in diverticular base, 50 cm from the ligament of Treitz		hemoclipping		1 month
Marangoni et al. 2009 [33]	UK	15/F	Melena, hematemesis	7.8	None	No	EGD showed a protruding vessel in the greater gastric curvature. CTA detected, but ex-lap confirmed the midjejunal lesion *	3	Anterior gastrotomy + jejunal resection	No	Recovered, 6 months
Saji et al. 2010 [49]	Japan	72/M	Temporary stupor	14.4	HTN	No	EGD and colonoscopy failed. CT and angiography identified the source in the proximal jejunum	4	TAE	Yes/ hemoclipping + sclerotherapy	Recovered
Jones et al. 2010 [55]	USA	76/F	Admitted for chemotherapy	8.3	AF, AML, chemotherapy	NR	EGD and colonoscopy failed. Blood scan showed jejunal bleeding. Embolization failed but push enteroscopy diagnosed the proximal jejunal lesion	5	Epinephrine + endoscopic hemoclipping	No	Recovered
Chen et al. 2010 [87]	China	80/M	Melena	11	NR	NSAIDs	CT and angiography failed. Barium showed diverticula. Peroral DBE showed Dieulafoy-like lesion on the fold in the proximal jejunum	4	Endoscopic hemoclipping	Yes/ surgery	Recovered, 9 months
Chen et al. 2010 [87]	China	81/F	Hematochezia	10.9	NR	No	CE showed diverticular bleeding. Barium study showed diverticula. Angiography was negative. Peroral DBE showed Dieulafoy-like lesion on the fold in the proximal jejunum	4	Endoscopic hemoclipping	No	Recovered, 8 months
Chen et al. 2010 [87]	China	73/M	Melena	10.7	NR	No	Peroral DBE showed Dieulafoy-like lesion on the fold in the proximal jejunum	1	Injection therapy + hemoclipping	No	Recovered, 9 months
Chen et al. 2010 [87]	China	74/F	Melena	9.6	NR	No	CT negative. CE and barium diverticular. Angiography negative. Peroral DBE showed Dieulafoy-like lesion on the fold in the proximal jejunum	5	Endoscopic hemoclipping	No	Recovered, 9 months
Min et al. 2010 [67]	Korea	37/M	Hematochezia	NA	NR	No	Initial CT unremarkable. Repeat CE picked and PE identified but ex-lap confirmed the proximal jejunal lesion	5	Surgical resection	No	Recovered
Kanth et al. 2010 [80]	USA	79.4 ±11.8/ 1M,1F	Melena, hematemesis, hematochezia	8.5 ± 2.3	HTN, CAD	Aspirin	Endoscopy diagnosed jejunal lesions in both the cases	2	Heater probe; epinephrine + hemoclipping	No	Recovered, 40.4 ± 35.8

Abbreviations: *, histopathologic confirmation of a Dieulafoy's lesion; AOC/NSAIDs, anticoagulants/non-steroidal anti-inflammatory drugs; UGI, upper gastrointestinal; ex-lap, exploratory laparotomy; NR, not reported; BRBPR, bright-red blood per rectum; IOE, intraoperative enteroscopy; SMA, superior mesenteric artery; EGD, esophagogastroduodenoscopy; HTN, hypertension; HOCM, hypertrophic obstructive cardiomyopathy; CHF, congestive heart failure; TB, tuberculosis; GIB, gastrointestinal bleeding; RH, right haemicolectomy; JSR, jejunal surgical resection; D & C, dilatation and curettage; DUB, dysfunctional uterine bleeding; LOC, loss of consciousness; PE, push enteroscopy; CE, capsule enteroscopy; APC, argon plasma coagulation; AF, atrial fibrillation; AML, acute myeloid leukemia; DBE, double-balloon enteroscopy; CAD, coronary artery disease.

Supplementary Table 2. Demographic, clinical presentation, comorbid conditions, lesion site, treatment, and outcome data of patients with rectal Dieulafoy's lesion (2011-2020).

Authors, year	Country	Age/ gender	Clinical presentation	Hemoglobin (g/dL)	Comorbid conditions	AOC/ NSAIDs	Diagnostic workup	Diagnostic attempts	Treatment	Rebleed/ treatment	Outcome, follow-up duration
Dulic-Lakovic et al. 2011 [74]	Austria	80/F	Obscure-overt GIB	NR	NR	NR	DBE was diagnostic for proximal jejunal lesion	1	Injection therapy + hemoclipping	No	Recovered, 7 months
Dulic-Lakovic et al. 2011 [74]	Austria	75/F	Obscure-overt GIB, TDA	NR	NR	NR	DBE identified jejunal lesion for proximal jejunal lesion	1	APC	Yes/ surgery	Recovered, 24 months
Dulic-Lakovic et al. 2011 [74]	Austria	71/M	Obscure-overt GIB, TDA	NR	NR	NR	DBE detected jejunal lesion for proximal jejunal lesion	1	Injection therapy + hemoclipping	No	Recovered, 14 months
Dulic-Lakovic et al. 2011 [74]	Austria	51/F	Obscure-overt GIB, TDA	NR	NR	NR	DBE was diagnostic for proximal jejunal lesion	1	Endoscopic hemoclipping	No	Recovered, 49 months
Dulic-Lakovic et al. 2011 [74]	Austria	77/F	Obscure-overt GIB, TDA	NR	NR	NR	DBE detected jejunal lesion for proximal jejunal lesion	1	Endoscopic hemoclipping	No	Recovered, 15 months
Dulic-Lakovic et al. 2011 [74]	Austria	35/M	Obscure-overt GIB, TDA	NR	NR	NR	DBE was diagnostic for proximal jejunal lesion	3	Endoscopic hemoclipping	No	Recovered, 11 months
Dulic-Lakovic et al. 2011 [74]	Austria	73/M	Obscure-overt GIB, TDA	NR	NR	NR	SBE detected the proximal jejunal lesion	2	APC	No	Recovered, 17 months
Dulic-Lakovic et al. 2011 [74]	Austria	75/F	Obscure-overt GIB, TDA	NR	NR	NR	SBE was diagnostic for proximal jejunal lesion	2	Sclerotherapy + APC	Yes/ surgery	Recovered, 3 months
Dulic-Lakovic et al. 2011 [74]	Austria	82/F	Obscure-overt GIB, TDA	NR	NR	NR	SBE identified the proximal jejunal lesion	1	Endoscopic hemoclipping	No	Recovered, 5 months
Eddi et al. 2011 [98]	USA	63/F	Melena	10.3	HTN, asthma, osteoarthritis, Billroth II gastrojejunostomy	Misoprostol/ diclofenac sodium	EGD, CT, and selective CTA failed. PE with pediatric colonoscope identified the lesion in the afferent loop of the jejunostomy	4	Epinephrine + hemoclipping + India-ink tattooing	No	Recovered
Chung et al. 2011 [85]	Taiwan	M	Melena, dizziness	6.6	None	No	EGD, colonoscopy, and CTA failed. SBE showed several jejunal diverticula and one eroded exposed mid-jejunal vessel was noted after performing irrigation on the reddish spot	4	Endoscopic hemoclipping	No	Recovered 3 months
Vakil et al. 2011 [40]	USA	21/M	Melena	11.4	Obesity	No	CT abdomen negative, emergency endoscopy and sigmoidoscopy negative. Mesenteric angiogram indicated but ex-lap confirmed jejunal lesion*	5	Surgical resection	No	Recovered
Paliwal et al. 2011 [111]	India	20-73 3F; 1M	Obscure-overt GIB	NR	NR	NR	CE 3; SBE 1	1 (3) 2 (1)	Hemoclipping (2), APC (1), surgical resection (1)	No	Recovered, 4-12 months
Landaeta et al. 2013 [79]	Venezuela	15-80 8F; 9M	Obscure-overt GIB, 17; shock, 14	NR	NR	NR	SBE/DBE (17); 12 lesions by oral approach and 5 were diagnosed by anal route	1 (13) 2 (4)	Injection (1), injection + APC (4), Injection + clips (7), clips (4), APC (1)	No	Recovered, 8-14 months
Prachayakul et al. 2013 [88]	Thailand	18-89 2F; 3M	Obscure-overt GIB	NR	NR	NR	SBE found lesions in mid-to-distal jejunum	4 (3) 2 (2)	Hemoclipping (3), Epinephrine (1) APC (1)	No	Recovered, 4 months
Johnson et al. 2013 [18]	USA	73/F	Hematochezia	NR	Abdominal aortic aneurysm, recurrent obscure-overt GIB	NR	EGD, colonoscopy, CE, 4 CTAs, ex-lap, SBE, tagged red cell scan, and aortogram were non-diagnostic. She died eventually. Autopsy diagnosed jejunal DL	-	-	-	Died
Kirschberg et al. 2013 [62]	Germany	NR	Melena, dizziness, sweating	9.3	Gastric cancer, Roux-en-Y gastric bypass, laparoscopic cholecystectomy, aneurysma spurium of the arteria hepatica dextra	NR	Upper endoscopy identified the lesion	1	Ethoxysclerol + hemoclipping	No	Recovered
Khalaf et al. 2014 [94]	USA	83/F	Epigastric pain, melena	9.1	HTN, GERD, DD, IDA, treated jejunal AVMs	NR	Pediatric colonoscope showed an actively bleeding arterial vessel in proximal jejunum	1	Endoscopic hemoclipping	No	Recovered

Kozan et al. 2014 [41]	Turkey	21/F	Syncope, hematochezia	3.7	PCOS	No	Angiography revealed bleeding from one of the proximal jejunal arteries. Ex-lap confirmed the lesion at 30 cm distal to the LT *	2	TAE	Yes/ surgery	Recovered
Lipka et al. 2015 [73]	USA	86/M	Melena	7.4	CABG, HTN, AF with PPM, CA bladder, gastritis	Aspirin, coumadin	EGD, colonoscopy, CE, and IR failed. SBE detected active jejunal bleeding	5	Bipolar + APC + hemoclipping	No	Recovered, 12 months
Lipka et al. 2015 [73]	USA	73/F	Melena	NR	NR	NR	EGD, colonoscopy, and CE failed. Repeat SBE detected proximal jejunal spurting	5	Bipolar	No	Recovered, 19 months
Lipka et al. 2015 [73]	USA	63/M	Melena	5.5	MI, CABG, CHF, AF, HTN, AS, CVA, OSA, alcoholism	Aspirin, clopidogrel	EGD, colonoscopy, and CE failed. Active bleeding on repeat SBE in the proximal jejunum	5	Bipolar + APC + endoscopic hemoclipping	No	Recovered, 18 months
Lipka et al. 2015 [73]	USA	81/M	Melena	6	CABG,CHF, AVR, PPM, AF, HTN, alcoholism	Aspirin, clopidogrel	EGD, colonoscopy, and CE failed. Active proximal jejunal bleeding on SBE	4	Bipolar	No	Recovered, 8 months
Lipka et al. 2015 [73]	USA	60/F	Melena	7	PUD, anemia, COPD	No	EGD, colonoscopy, CE, and PE failed. Oozing on repeated SBE in the proximal jejunum	6	Bipolar + hemoclipping	Yes/ hemoc lipping	Recovered, 1.5 months
Lipka et al. 2015 [73]	USA	60/F	Melena	8.3	MI, CHF, HTN, HLD, DM, Anemia	Aspirin	EGD, colonoscopy, and CE failed. Active proximal jejunal bleeding on SBE	4	APC	No	Recovered, 15 months
Lipka et al. 2015 [73]	USA	80/M	Melena	5.3	MI, CHF, AS, MR/ TR, Afib, HTN, CVA	Aspirin, clopidogrel	EGD, colonoscopy, and CE failed. Active bleeding on repeat SBE in the proximal jejunum	5	Epinephrine + Bipolar	No	Recovered, 44 months
Ego et al. 2015 [90]	Japan	95/F	Melena	4.3	CHF, CKD	NR	EGD and colonoscopy failed. DBE detected upper jejunal lesion	3	Endoscopic band ligation	No	Recovered, 1 month
Bahar et al. 2015 [48]	USA	78/M	Melena	NR	CAD, AF, ESRD on HD	NR	EGD and colonoscopy failed. CE showed small bowel blood. DBE revealed a bleeding Dieulafoy's approximately 240 cm from the pylorus	4	Epinephrine + endoscopic hemoclipping	No	Recovered
Bahar et al. 2015 [48]	USA	64/M	Melena	NR	CHF, DM2, HTN	NR	EGD and colonoscopy failed. CE showed small bowel blood. DBE revealed a bleeding Dieulafoy's approximately 110 cm from the pylorus	4	Epinephrine + endoscopic hemoclipping	No	Recovered
Sousa et al. 2016 [47]	Portugal	25/M	Hematochezia	3.8	HIV, GIB, rectal polyp excision	No	CTA detected jejunal bleeding. IOE on ex-lap showed a mid-jejunal 1 cm-pseudopolypoid lesion with surface hyperemia and fresh adherent clot was found *	2	Surgical resection	No	Recovered
Chao et al. 2016 [70]	Taiwan	15/M	Hematochezia, hypotension	8.1	None	No	EGD, colonoscopy, and DBE failed. CTA detected jejunal lesion. Ex-lap confirmed distal jejunal lesion, 125 cm from the Triet'z ligament	5	Surgical resection	No	Recovered
Holleran et al. 2016 [56]	Ireland	67/F	Obscure-overt GIB	NR	RF, aortic and MVR, CHF, cirrhosis	Yes	EGD, colonoscopy, CTA, and CT failed. CE showed jejunal blood. DBE detected protruding proximal jejunal vessel	6	APC + endoscopic hemoclipping	Yes/ APC + clips	Died due to cardiac complication s
Holleran et al. 2016 [56]	Ireland	74/F	Melena	NR	RF, MVR, CHF, SBAD, cirrhosis, portal HTN, cholecystitis	Yes	EGD and colonoscopy failed. CE showed active bleeding and a minute mucosal defect in her proximal jejunum on DBE	4	APC	No	Recovered, 24 months
Holleran et al. 2016 [56]	Ireland	76/F	Melena	NR	MVR, CML, cirrhosis, HTN, jejunal AE (TAE)	Yes	SBSE detected bleeding but no focus. CTA failed. DBE identified a bleeding focus in proximal jejunum *	3	Surgical resection	No	Recovered, 9 months
Aoyama et al. 2016 [50]	Japan	79/M	Exertional dyspnea, melena, anemia	8.3	Angina pectoris	Aspirin, clopidogrel	EGD, colonoscopy, and CT failed. CE showed jejunal bleeding. DBE confirmed proximal jejunal pulsatile bleeding source	5	APC + endoscopic hemoclipping	No	Recovered
Lee et al. 2016 [101]	Korea	38/F	Abdominal pain, hematochezia	10.4	None	No	CT showed active jejunal bleeding. SMA angiography detected bleeding 3rd jejunal branch. IOE confirmed small exposed jejunal vessel, 40 cm distal to Treitz ligament *	3	TAE	Yes/ surgery	Recovered, 24 months
Seo et al. 2017 [102]	Korea	25/M	Hematochezia	13.8	None	No	EGD and colonoscopy failed. Angiography identified the culprit mid-jejunal artery. CT ruled out tumor, CE indicated rebleed, SBE identified recurrence. Minimal lap confirmed the lesion*	7	TAE	Yes/ surgery	Recovered, 24 months
Sinha et al. 2018 [20]	USA	55/M	BRBPR, hypotension	10.6	Alcoholic liver cirrhosis	No	EGD, colonoscopy, and CTA failed. PE identified jejunal culprit vessel with normal surrounding mucosa	4	Epinephrine + endoscopic hemoclipping	No	Recovered
Jung et al. 2019 [92]	Germany	69	Hematemesis, melena, fatigue, anemia	6.9	Gastric + cecal angiodysplasia	No	EGD failed. SBE identified a Dieulafoy-like pulsating lesion in the distal jejunum	2	Endoscopic hemoclipping + OTSC	No	Recovered
Cardoso et al. 2019 [75]	Portugal	21/M	Fatigue, dizziness	3.4	Type 1 vWD, gastrduodenal	No	EGD and colonoscopy failed. CE identified jejunal blood but push	4	Adrenalin + hemoclipping +	No	Recovered, 18 months

					DL, chronic IDA		enteroscopy detected the lesion in the proximal jejunum		tattooing		
Zhao et al. 2019 [61]	China	41/M	Hematochezia, syncope	NR	None	No	EGD and colonoscopy failed. SBE detected and ex-lap confirmed a proximal jejunal GIST-like lesion harboring DL*	4	Surgical resection	No	Recovered, 6 months
Garg et al. 2019 [59]	India	34/M	Melena, giddiness, weakness	4.6	Hemorrhoids		EGD and colonoscopy normal. CT showed IC junction thickening. Repeat EGD picked midjejunal DL	5	Injection sclerotherapy	No	Recovered
Saada et al. 2019 [43]	Israel	27/M	Abdominal pain, dysentery, syncope	13	None	No	EGD, colonoscopy, sigmoidoscopy, and angiography failed. Red cell scan indicated small bowel bleeding. Ex-lap confirmed jejunal bleeding site. *	5	Surgical resection	No	Recovered, 3 months
Kawabata et al. 2019 [21]	Japan	81/M	Fever, edema, hemosputum, dyspnea, oliguria, macrohematuria	6.3	CVA, GS	Aspirin + cilostazol	CT detected and per oral enteroscopy showed a DL with gush-out hemorrhage from a protruding visible vessel on the jejunal wall 40 cm from the ligament of Treitz	2	Endoscopic hemoclipping	No	Died after 10 weeks due to worsening of GS
Mendo et al. 2020 [42]	Portugal	74/M	Hematemesis, hematochezia	8.8	Chronic AF	Dabigatran	Repeat EGD failed. CTA detected but ex-lap confirmed a bleeding jejunal diverticulum, 30 cm distal to the ligament of Treitz *	4	Surgical resection	No	Recovered
Oladunjoye et al. 2020 [68]	USA	80/F	Melena, dyspnea on exertion, body weakness	NR	NR	No	EGD failed PE identified jejunal lesion	2	APC + hemoclipping	No	Recovered
Yano et al. 2020 [93]	Japan	68/M	Light headedness	5.6	Gastric CA (distal gastrectomy and Bilroth I), brain infarction	Dabigatron	EGD identified the jejunal lesion	1	Endoscopic hemoclipping	Yes/ OTSC + banding	Recovered
Pérez Fernández et al. 2020 [97]	Spain	55-86; median: 71/M6, F7	Melena 10, hematochezia 1	NR	HTN 10, VHD 5	Yes (10)	CE identified lesion in 2/13 while DBE diagnosed 10 patients and 1 had no endoscopic findings	4 (13)	APC + epinephrine (8), APC + epinephrine + hemoclipping (3); APC + hemoclipping (1); epinephrine + hemoclipping (1)	Yes/ APC + epinephrine 2; surgery 1	Recovered, 2-145; median 33.5 months
Yehya et al. 2020 [108]	USA	21/M	BRBPR, hematemesis	10	None		CTA, EGD, and colonoscopy failed. Blood scan indicated jejunal lesion but ex-lap confirmed a proximal jejunal lesion *	4	Surgical resection	No	Recovered
Jaiswal et al. 2020 [109]	USA	21/M	Hematochezia, shock	13.5	None	No	CTA failed. IOE showed jejunal lesion	2	Surgical ligation	No	Recovered

Abbreviations: *, histopathologic confirmation of a Dieulafoy's lesion; AOC/NSAIDs, anticoagulants/non-steroidal anti-inflammatory drugs; GIB, gastrointestinal bleeding; NR, not reported; DBE, double-balloon enteroscopy; TDA, transfusion-dependent anemia; APC, argon plasma coagulation; SBE, single-balloon enteroscopy; HTN, hypertension; PE, push enteroscopy; CE, capsule endoscopy; Ex-lap, exploratory laparotomy; GERD, gastroesophageal reflux disorder; DD, diverticular disease; IDA, iron-deficiency anemia; AVM, arteriovenous malformation; IOE, intraoperative enteroscopy; PCOS, polycystic ovary syndrome; TAE, transcatheter arterial embolization; CABG, coronary artery bypass grafting; AF, atrial fibrillation; CA, carcinoma; IR, interventional radiology; MI, myocardial infarction; CHF, congestive heart failure; AS, aortic stenosis; CVA, cerebrovascular accident; OSA, obstructive sleep apnea; AVR, aortic valve replacement; PPM, permanent pacemaker; PUD, peptic ulcer disease; COPD chronic obstructive pulmonary disease; HLD hyperlipidemia; DM, diabetes mellitus; MR, mitral regurgitation; TR, tricuspid regurgitation; DL, Dieulafoy's lesion; ESRD, end-stage renal disease; RF, rheumatic fever; MVR, mitral valve replacements; SBAD, small bowel angiodysplasia; CML, chronic myelogenous leukemia; BRBPR, bright-red blood per rectum; vWD, von Willebrand disease; IC, ileocecal; GS, Goodpasture syndrome; VHD, valvular heart disease.